

## Resumé

Samtalebaseret kunstig intelligens er på få år blevet en del af mange menneskers hverdag. Værktøjer som ChatGPT bruges ikke kun til studie- og arbejdsrelaterede opgaver, men også til spørgsmål om sundhed, trivsel og personlige bekymringer. Man kan spørge om alt fra diverse fysiske symptomer til søvnproblemer, stress, usikkerhed, eller tanker man ikke nødvendigvis ønsker at dele med andre mennesker. Selvom denne brug er blevet mere almindelig, ved vi stadig relativt lidt om, hvordan mennesker faktisk oplever disse interaktioner i praksis. Dette speciale undersøger derfor, hvordan folk oplever conversational AI, når de oplever fysiske eller mentale sundhedsrelaterede bekymringer.

Undersøgelsen bygger på 20 semistrukturerede interviews med folk, som tidligere havde brugt conversational AI til helbredsrelaterede spørgsmål. Som en del af interviewene gennemførte deltagerne en scenariebaseret chatbot-interaktion, hvor de skulle snakke med ChatGPT om enten en fysisk- eller mental trivselssituation. Interviewmaterialet blev behandlet gennem refleksiv tematisk analyse. Det vil sige, at analysen var en fortolkende proces, hvor forfatterens egne tilgangsvinkler som forskere også havde betydning. Forfatterne arbejdede med gentagende runder af gennemlæsning, kodning, fælles diskussioner og refleksive noter, for at udføre analysen. Forfatterne brugte løbende disse diskussioner og refleksive noter til at forstå hvordan deres egne forståelser af AI, HCI, og undersøgelsens tematikker påvirkede analysen.

Gennem analysen blev skabt fire temater: *Tilgængelighed og Lav Indsats, Selvforståelse og -Udvikling, Bruger Agens og Sociale Kontrakter og Privatliv*. Analysen viste først og fremmest, at deltagerne ofte brugte chatbots som en let tilgængelig start på at undersøge en bekymring. Mange beskrev, hvordan det føltes hurtigere og nemmere at spørge en chatbot, frem for at søge gennem hjemmesider eller involvere andre mennesker. For flere var det nærmest blevet en automatisk vane at åbne ChatGPT, når de stødte på noget usikkert. For mange fungerede chatbots som et redskab til at skabe overblik og mening i situationer, hvor deres problem endnu ikke var klart defineret. Deltagerne brugte chatbots til at samle symptomer, tanker og bekymringer i én samtale og få hjælp til at forstå, hvad der kunne være relevant at undersøge nærmere. I flere tilfælde hjalp chatbotten med at formulere indholdet af spørgsmålene, før den hjalp med svarene. Et andet resultat var opfattelsen af kontrol og agens. Deltagerne beskrev, hvordan de selv kunne styre samtalen, vælge hvilke oplysninger de ville dele, og beslutte om chatbottens svar var tilstrækkelige, eller om de ønskede at undersøge sagen videre med andre kilder. Denne frihed blev ofte oplevet som en fordel, men betød også at ansvaret for at vurdere svarenes kvalitet lå hos brugeren selv. Derudover viste analysen, at chatbots for mange deltagere gav en anderledes socialt stemning end samtaler med andre mennesker. Her kunne de stille spørgsmål uden at bekymre sig om at blive dømt, misforstået eller gøre andre mennesker bekymrede på deres vegne. Dette var især tydeligt ved mere følsomme eller personlige emner. Samtidig varierede deltagernes forventninger, afhængigt af typen af bekymring. Ved fysiske symptomer ønskede de ofte konkrete og handlingsorienterede svar, mens mentale trivselsspørgsmål i højere grad gjorde anerkendelse, refleksion og støtte vigtig.

På tværs af alle temaerne spillede tillid en vigtig rolle. Deltagerne satte pris på svar, der virkede relevante, personlige og beroligende. Men netop disse egenskaber kunne også skabe usikkerhed. Flere deltagere beskrev, hvordan chatbotten nogle gange kunne virke for enig eller for bekræftende. Tillid handlede om, hvorvidt svaret føltes passende i den konkrete situation.

Specialet bidrager med en empirisk forståelse af conversational AI i konteksten af fysisk- og mental velvære. Resultaterne peger på, at chatbots ofte fungerer som meningsskabende redskaber, der hjælper brugere med at håndtere og strukturere deres usikkerheder omkring helbred.

# "I'll just ask ChatGPT..."

Conversational AI as a sense-making tool for physical and mental well-being

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## Abstract

Conversational AI is increasingly used for everyday physical and mental well-being concerns, yet not a lot is known about how people experience and make sense of these interactions in practice. This study explores how people use and perceive conversational AI when managing well-being concerns. We conducted semi-structured interviews with 20 participants who had previously used conversational AI for health-related questions or concerns. As part of the interviews, participants completed a scenario-based chatbot task. The interview material was analysed using reflexive thematic analysis. Our findings suggest that participants often used chatbots as accessible and low-friction tools for making sense of uncertainty. Chatbots were used to explore symptoms, thoughts, and concerns before they were fully defined, helping participants organise information, consider possible explanations, and decide on next steps. Participants valued the speed, availability, privacy, and flexibility of chatbot interactions, while also describing tensions related to verification, reassurance, responsibility, and trust. We contribute an empirical account of conversational AI as a sense-making tool in everyday well-being practices and show how the qualities that make these systems useful can also shape how concerns are understood and acted upon.

## Keywords

Conversational AI, Well-being, Sense-making, Trust, User Agency, Human-AI Interaction

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## 1 Introduction

Conversational AI has quickly become part of everyday information-seeking. General-purpose chatbots such as ChatGPT are used for writing, planning, problem-solving, and increasingly also for questions related to health and well-being. These uses are not limited to formal medical advice. They also include smaller and more everyday concerns, such as unclear symptoms, sleep, fitness, medication, anxiety, stress, or the need to understand why one feels a certain way.

Conversational AI changes the form of health information-seeking. With search engines, users often have to translate their concern into searchable terms and compare results across sources [37, 41]. With chatbots, users can describe symptoms, feelings, context, uncertainty, and follow-up questions in the same interaction [23, 39, 41]. This makes chatbot interaction relevant when users do not yet know how to define the concern they are trying to understand [8, 24]. At the same time this creates new risks and responsibilities. Chatbot responses can feel efficient, structured, and personally relevant. They may also offer reassurance or a sense of being understood. This can be helpful in well-being contexts, but also ethically complex, especially when the concern is sensitive, emotionally loaded or difficult for the user to evaluate. Users may need to decide whether

to trust, check, ignore, continue, or act on the response, often while already feeling uncertain.

The relevance of this topic is also shaped by the wider adoption of generative AI. A survey shows that 73% of Danes who had used generative AI within the last three months reported using it for personal purposes [32]. At the same time, public trust in AI for health information remains limited. A survey on health misinformation found that only a minority of respondents trusted AI to provide reliable health information either 'a fair amount' or 'a great deal' [14]. This tension between increasing use and uncertain trust is central in our interest in everyday chatbot use for health and well-being.

Prior work in Large Language Model (LLM)-based health information-seeking shows that chatbot use creates new user work around prompting, interpreting the responses and deciding how much users rely on the output [24, 41]. Chatbot responses also feel efficient and personally relevant because they can be immediate, structured, and acknowledging of the specific context [30]. This suggests that chatbots not only provide information. They may also offer reassurance or a sense of being understood, which makes their role in well-being contexts both useful and ethically complex [15].

This study explores how people make use of and perceive conversational AI when managing physical and mental well-being concerns. We conducted semi-structured interviews with 20 participants who had previously used conversational AI for a health-related question or concern. The interviews included questions about participants' everyday chatbot use, their preferred sources of health information, and their experiences with chatbot responses. Participants also completed a scenario-based chatbot task, where they wrote to a chatbot about either a physical or mental well-being concern as they normally would. The full interviews were analysed using reflexive thematic analysis.

We adopt a holistic view of well-being. Physical concerns include bodily symptoms, sleep, exercise, and general health questions. Mental well-being concerns include stress, anxiety, reassurance, and self-reflection. Our focus is on the informal role chatbots may play before, after, or alongside other sources of help, such as search engines, health websites, family, friends, and healthcare professionals. In this paper, we use terms such as generative AI, chatbots, AI, and 'the chat' pragmatically. Participants often used these terms interchangeably, and the technical differences were not always meaningful for how they described their experiences. We therefore distinguish between them only when relevant to the analysis.

The paper is guided by the following research question: *How do people use and perceive conversational AI when managing physical and mental well-being concerns?*

By addressing this question, we contribute an empirical account of conversational AI as a sense-making tool for everyday well-being uncertainty. We show that participants use chatbots as low-friction spaces for making unclear symptoms, feelings, and concerns more understandable. Across the analysis, low-friction access, user agency, privacy, reassurance, and verification were all part of this broader sense-making process. We argue that the value and risk of chatbot-supported well-being practices lie in this same quality: chatbots can make uncertainty feel manageable, while also shaping

which interpretations, concerns and next steps surface and become clear.

## 2 Related Research

This section positions our study in relation to three areas of prior work, before identifying the research gap that motivates our study. First, we discuss conversational AI as part of everyday health information-seeking, where users use chatbots alongside search engines, health websites, forums, and social media. Second, we discuss work on reassurance, emotional support, and self-disclosure, since well-being concerns are not always only informational. Third, we discuss boundaries, trust, responsibility, and user strategies, as chatbot use requires users to interpret, verify, and manage the interaction. Finally, we describe the research gap we address in this study.

### 2.1 Conversational AI and Health Information-Seeking

Online health information-seeking is not new. People have long used the internet to look up symptoms, understand health concerns, and decide whether to seek professional help [21]. Conversational AI changes the form of this information-seeking. With search engines users often have to reduce a concern down to a few searchable terms [37, 41]. With chatbots, users can describe symptoms, context, uncertainty, and follow-up questions in the same interaction [23, 39, 41].

Recent work shows that LLMs are becoming a part of this wider health information domain. Yun and Bickmore show that LLM-based chatbots are used alongside search engines, health websites, forums, and social media, and that users do not necessarily evaluate these sources in the same way [41]. Ramesh et al. describe the use of off-the-shelf conversational agents for health information as a form of user work, where people must formulate prompts, interpret responses and decide how much they can rely on the output [24]. Similarly, Shahsavari and Choudhury show that users' intentions to use ChatGPT for self-diagnosis is shaped by the perceived usefulness, risk-reward evaluation and different expectations around decision-making [27].

Health information is not only about the access, but also about interpretation. Joshi et al. found that users of symptom diagnosis chatbots value clear explanations, structured dialogue, consistency, transparency, and concrete next-step guidance [11]. Mingole et al. evaluated LLM responses to everyday health prompts, and found that many responses were considered accurate by physicians, while still pointing to possible risks around harm, over-reliance and inappropriate use in self-diagnosis [18]. Zhang et al. further show that online self-diagnosis can be shaped by availability bias, where exposure to health-related information can make people overestimate their own symptoms [42].

Together, these works show that conversational AI is becoming part of everyday health information practices. It may help users access information quickly and structure their uncertain concerns, but it also places responsibility on users to judge relevance, credibility, risk, and whether professional help is needed.

## 2.2 Reassurance and Emotional Support

Health-related chatbot use is not only about finding information. Several studies show that people also use chatbots for reassurance, emotional support and personal reflection. Skjuve et al. compared young people’s evaluations of responses to mental health questions written by ChatGPT and by professionals from the Norwegian youth information service ung.no. Their study found that participants were generally positive towards both, but more often recommended the ChatGPT responses to others. In their qualitative findings, participants pointed to the information quality, acknowledgement, and well-structured answers as reasons for their preferences [30]. Jung et al. also show that people discuss mental health issues with ChatGPT and describe receiving validation, actionable advice, and help preparing for therapy, but while raising concerns about incorrect information, privacy, and validation of users from the system [12].

Other work has examined LLM use in more emotionally sensitive contexts. Purohit and Heuer show that people with mental health challenges may value immediacy, non-judgement, self-paced disclosure, cognitive reframing, and relational engagement. At the same time, their participants also described clear boundaries, perceiving LLMs as more useful for mild-to-moderate distress than for crises, trauma, or complex social-emotional situations [22]. Zheng et al. describe emotional support in this broader sense and show how users can shape LLM-powered chatbots through personas and interaction styles to better fit their needs [43]. These interaction dynamics are rooted in self-disclosure. Earlier work on online disinhibition shows that people may disclose more online than they would face-to-face, partly because of anonymity, invisibility, and distance [33]. Cox et al. show a similar issue in chatbot interaction. In their study a chatbot framed as a stranger sometimes made emotional disclosure feel easier because it provided anonymity and reduced judgement, and a familiar chatbot could be experienced as intrusive if rapport had not already been built [5].

Recent Danish research also points to this kind of everyday emotional use. VIVE’s qualitative study describes young people’s use of ChatGPT for social and emotional support and examines how they understand its role in relation to mental well-being and existing supportive offers [13]. Wester et al. add a perspective by focusing on conversational care. In their study with chaplains, they argue that care is not only about giving advice or solving a problem, but also about listening, connecting, carrying, and being present with someone through difficulties [38]. Together these works show why emotional support through AI is difficult to understand solely as informational.

## 2.3 Sense-making and AI-Supported Interpretation

Sense-making is useful for understanding chatbot use, because well-being concerns are not always clearly formed before people seek information. In HCI, sense-making has been used to describe how people organise complex information into structures that support interpretation and action. Gero et al. show that users may need support in comparing, organising, and interpreting multiple LLM outputs, since generated responses can themselves become material that users must make sense of [8].

This perspective is relevant to health-related chatbot use because conversational AI does not only retrieve information. It can also suggest categories, possible explanations, seriousness levels, and next steps. In professional health-related contexts, Mastrianni et al. describe sense-making as part of complex clinical knowledge work and examine how generative AI might support professionals in working through difficult information [17]. Farrell et. al. go one step further and analyse chatbots as a social technology that helps with navigating large amounts of complex information [7]. Chatbots become a tool for making sense of complex information, as they organise, synthesise and reconstruct information into more narrow and focused results. With conversational sense-making, users can use prompting strategies to co-construct the context. As Xue et al. notes, this allows users to shape the chatbots for better interpreting their subjective experiences[39]. Our study examines a different setting, with everyday well-being concerns among non-professional users. Sense-making is less about analysing clinical data and more about making vague symptoms, feelings, or concerns understandable enough to decide what to do next.

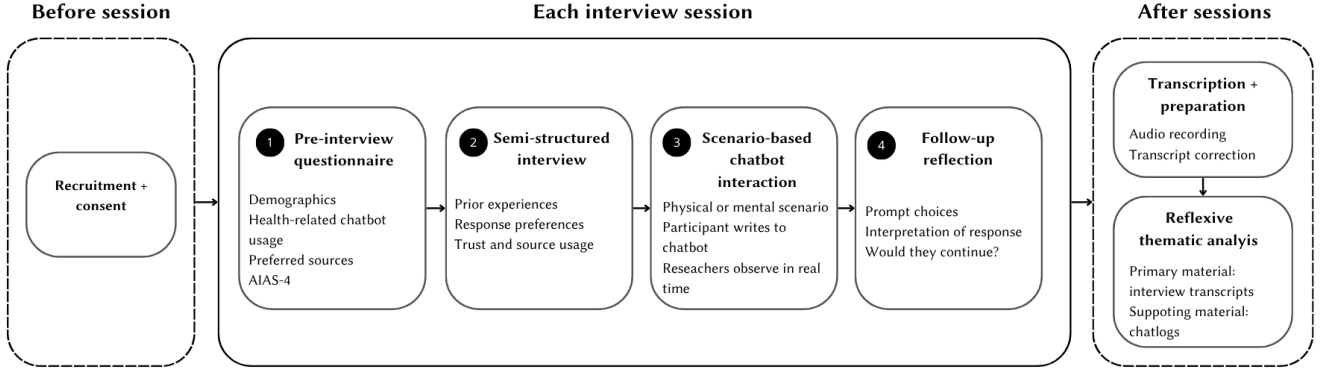
## 2.4 Boundaries, Trust and User Strategies

Because chatbot interactions unfold through conversation, users are not only passive recipients of information. They can test, compare, correct, continue, or stop the interaction. Noshin et al. show that users detect sycophantic behaviour through strategies such as cross-platform comparison and consistency testing, and respond through prompting techniques and other forms of language engineering [19]. This is relevant in health and well-being contexts, where users may want reassurance but also need responses that do not simply confirm what they already think is wrong.

Sycophancy is a broader concern in language models. Sharma et al. show that AI assistants can produce responses that match a user’s beliefs over truthful ones and that human preference data may reward responses that align with the user’s view [28]. In health and well-being contexts this may influence how users understand and act on their symptoms, feelings, or personal dilemmas.

Trust is also connected to agency and control. Yun et al. argue that agency in sustained human-AI conversation is not only controlled by either the user or the chatbot, but is negotiated in turns through initiative, boundaries, adaptation, and conversational steering [40]. This means that the shape of an interaction can matter as much as the final answer. In sensitive contexts, even safety mechanisms are experienced as part of the conversation. Tang et al. argue that refusals in mental health support interactions should not be understood as single-turn events, but rather as dynamic experiences shaped by expectations, refusal framing, referral resources and what happens after the refusal [34]. These studies show that health-related chatbot use depends on more than access to an answer. It also depends on how users manage the interaction, how they understand the chatbot’s role in relation to their own knowledge and how the system balances user support and uncertainty.

The use of general-purpose chatbots for well-being also raises questions about privacy, and responsibility. Kollig et al. argue that emotional support conversational AI can create risks when there are mismatches between what a system appears to offer, what it can technically provide, and how users actually use it [15]. This is



**Figure 1: Overview of the study procedure.** The workflow spans three parts: pre-session recruitment and consent; core interview session encompassing (1) pre-interview questionnaire, (2) Semi-structured interview, (3) Scenario-based chatbot interactions, and (4) Follow-up reflection; and after session data processing. Interview transcripts were the primary material for the reflexive thematic analysis, while chatlogs were used as contextual material.

particularly relevant for general-purpose chatbots, which are not designed as explicit health technologies, but are still used by people for physical and mental well-being concerns.

Work on AI and health-related stigma further shows why sensitive contexts require care. Song et al. find that AI can both reduce and reinforce health-related stigma, for example by offering perceived anonymity while also raising concerns about bias, privacy, and loss of trust [31]. Research in HCI emphasises that systems should be understood in relation to human well-being, privacy, responsible design, and users’ cognitive capacities [20]. This work on meaningful human control adds an important perspective here. Siebert et al. argue that responsibility attributed to a human should be proportionate to that person’s actual ability and authority to control the system [29]. In health-related chatbot use, users may be able to prompt, continue, stop, or verify an interaction, while still lacking the knowledge or certainty required to evaluate the response.

## 2.5 Gap Description

Existing research provides valuable insights into how people evaluate chatbot responses, how they manage trust and uncertainty, and how conversational AI may be used for emotional support or seeking health information. Much of this work relies on structured scenarios, predefined interaction stages, or evaluations of specific chatbot responses. For example, Ramesh et al. used detailed fictional personas to study metacognitive demands in health information-seeking with conversational agents [24]. Joshi et al. used predefined interaction stages: initial assessment, detailed inquiry, and diagnosis, to study user preferences in healthcare conversational AI [11]. These types of study designs make interaction more comparable across participants, but they can also structure and affect the users’ interactions. This leaves a need for qualitative work that examines how people formulate concerns, navigate responses, and make sense of chatbot interactions when they have room to draw on their own habits and prior experiences. Our study uses broad

physical and mental well-being scenarios as a shared point of departure, while still allowing participants to formulate the concern in their own way. In doing so, we examine how they approach the interaction, how they draw on previous experiences, and how they manage uncertainty as a conversation unfolds. Our focus is on user sense-making, how people communicate unstructured well-being concerns, and the strategies they use to manage health-related uncertainty through conversational AI.

## 3 Research Method

This study explores how people engage with conversational AI in relation to physical and mental well-being concerns. We used a qualitative interview design (Figure 1), where each participant took part in an interview session consisting of the four phases:

- (1) Pre-interview questionnaire: To capture participants’ background information and attitudes towards AI through the AI Affinity Scale (AIAS-4).
- (2) Semi-structured interview: A reflective phase where participants elaborated their motivations, sense-making processes and experiences with the chatbot.
- (3) Scenario-based chatbot interaction: A session where participants engaged with a chatbot in an open-ended dialogue about a predefined scenario.
- (4) Follow-up reflection: Evaluative phase of participants’ immediate prompt choices, interpretation of responses and continuation of interaction.

The questionnaire gave background context, the interview provided participants’ experiences and reflections, and the chatbot interaction gave us a concrete interaction to observe and discuss.

### 3.1 Participant Sampling and Recruitment

We recruited 20 participants with prior experience using conversational AI for a health-related question or concern. Participants were required to be at least 18 years old and able to speak either Danish or English. Recruitment was conducted through online posts,

**Table 1: Participant demographics, self-reported AI usage, and AIAS-4 score. Individual question columns (Q1–Q4) include horizontal progress bars representing participant rating out of 10. Summary rows at the bottom provide the median (Mdn) and interquartile range (IQR) for the aggregate scores and individual items.**

| ID  | Age | Gender     | Occupation            | Est. monthly use | AIAS-4 Score | Q1  | Q2  | Q3   | Q4  |
|-----|-----|------------|-----------------------|------------------|--------------|-----|-----|------|-----|
| P1  | 22  | Man        | Dairyman              | 10               | 10.0         | 10  | 10  | 10   | 10  |
| P2  | 25  | Woman      | University Student    | 10               | 7.0          | 6   | 7   | 8    | 7   |
| P3  | 22  | Man        | Nursing Student       | 10               | 6.3          | 6   | 3   | 9    | 7   |
| P4  | 25  | Woman      | University Student    | 3                | 8.8          | 9   | 8   | 10   | 8   |
| P5  | 25  | Woman      | University Student    | 3                | 6.3          | 4   | 5   | 10   | 6   |
| P6  | 24  | Man        | Software Engineer     | 2                | 6.5          | 3   | 8   | 10   | 5   |
| P7  | 34  | Woman      | University Student    | < 4              | 6.5          | 5   | 8   | 10   | 3   |
| P8  | 28  | Man        | University Student    | 2                | 8.6          | 7.5 | 10  | 10   | 7   |
| P9  | 27  | Woman      | On Parental Leave     | 15               | 9.3          | 10  | 10  | 10   | 7   |
| P10 | 25  | Woman      | University Student    | 4                | 7.5          | 5   | 7   | 10   | 8   |
| P11 | 30  | Woman      | Unemployed            | 3                | 6.0          | 6   | 6   | 7    | 5   |
| P12 | 33  | Woman      | Prompter              | 4                | 7.0          | 7   | 6   | 10   | 5   |
| P13 | 28  | Woman      | Pharmacy Technician   | 1                | 3.0          | 4   | 2   | 4    | 2   |
| P14 | 26  | Non-binary | Graphic Designer      | 0                | 5.8          | 5   | 6   | 8    | 4   |
| P15 | 22  | Woman      | Employed, Unspecified | < 1              | 5.5          | 6   | 3   | 7    | 6   |
| P16 | 21  | Woman      | University Student    | 1                | 6.5          | 4   | 7   | 10   | 5   |
| P17 | 24  | Trans-man  | University Student    | 4                | 4.1          | 3   | 1   | 9.5  | 3   |
| P18 | 24  | Woman      | University Student    | 7                | 8.5          | 6   | 8   | 10   | 10  |
| P19 | 27  | Man        | Caseworker            | 3                | 6.3          | 3   | 7   | 10   | 5   |
| P20 | 27  | Woman      | Pharmacy Technician   | 2                | 2.5          | 3   | 2   | 4    | 1   |
|     |     |            |                       |                  | Mdn          | 6.5 | 7.0 | 10.0 | 5.5 |
|     |     |            |                       |                  | IQR          | 2.5 | 4.5 | 2.0  | 2.7 |

flyers handed out on university campuses, and personal networks. Participants were entered into a raffle in which three participants each won a 100 DKK gift card. All interviews were conducted in the participant’s preferred language. Danish interview excerpts were later translated into English by the authors, with attention to preserving informal phrasing and intended meaning.

### 3.2 Data Collection

Our data collection took place either in person or through online platforms depending on participant preference. Before each interview, participants signed a consent form developed in accordance with AAU’s guidelines for responsible data handling. We considered what personal data we were responsible for, what information we needed, and how participants should be informed about the use, storage, and anonymisation of their data.

The interviews were audio recorded and transcribed using NoScribe and NoScribe Editor, which use a locally run version of Whisper to generate transcripts [6]. The generated transcripts were manually checked and corrected by the authors. The final transcripts formed the base material for the reflexive thematic analysis, as shown in Figure 1 (After sessions).

**3.2.1 Pre-interview Questionnaire.** Before the main interview, participants completed a short pre-interview questionnaire. It collected demographic information, their estimated monthly health-related chatbot use, preferred sources of health information, and general attitudes toward AI (see Table 1 for overview of collected data). The questionnaire also included the AI Affinity Scale (AIAS-4), a short four-item scale for measuring general attitudes toward artificial intelligence [9]. Participants answered each AIAS-4 item on a 1–10

scale, where higher values meant a more positive attitude toward AI.

We included the AIAS-4 to contextualise the interview material, allowing us to ground additional questions in the participants’ specific attitudes. Because the study focuses on how people interpret and evaluate conversational AI, participants’ broader attitudes toward AI were relevant background information. The scale helped us describe whether the participants entered the interviews with generally positive, negative, or neutral expectations toward AI. It also helped us interpret the interview material with more context, since reflections on trust, usefulness, and risk may be shaped by participants’ general orientation toward AI. Our sample was small and selected based on prior health-related chatbot use, so the scores should not be read as representative of a larger population.

The questionnaire was used descriptively, alongside the demographic information and interview data, and it helped us tailor follow-up questions during the interview. For example, if a participant reported frequent health-related chatbot use, we asked them to describe recent examples. If they reported using other sources, such as specific health websites or search engines, we asked how those sources differed from their use of chatbots in similar situations.

**3.2.2 Semi-Structured Interviews.** We conducted 20 semi-structured interviews, each lasting approximately 45 minutes, grounded in the qualitative framework on approaching interviews, proposed by Hornbæk et al. [10]. We focused on three main areas: motivations for chatbot use, preferences for response styles, and perceptions of successful outcomes. This included questions about trust, tone, accuracy, reassurance, and their chatbot-use compared to other sources such as search engines, family, friends, or healthcare professionals. The semi-structured format allowed us to maintain a

consistent focus throughout all 20 interviews, while allowing space for participants to introduce experiences, concerns, and reflections that emerged during each session.

To keep consistency throughout our data collection, we developed a two-page interview guide to support the interviews (see Appendix A.1). The guide included our main areas of interest, example questions for each topic, and the reason for including each topic. This made the guide more flexible rather than instructive. The interviewers could reformulate the questions, ask follow-ups, or change the order of questions depending on how the conversation developed. The interview guide also had a list of keywords and topics from the existing research, so if those topics were to emerge during the interviews, the interviewers would be able to remind themselves to ask further questions. This follows the role of open-ended interviews in Human Computer Interaction (HCI), where the interviewer works from a prepared focus but adapts questions and follow-ups as the conversation develops [10].

**3.2.3 Chatbot interaction, scenarios and follow-up reflection.** The scenario-based chatbot task and subsequent follow-up reflection were included as part of the interview. We used it for four reasons. First, everyday chatbot use is often private and difficult to recall in detail. Second, we did not want to ask participants to show us their own previous health-related chatbot conversations as they could contain sensitive personal information. Third, the scenarios gave participants a shared starting point, making the interviews more comparable. Finally, the task allowed us to observe how participants formulated a concern, selected specific details, read the response, and decided whether they wanted to continue the interaction.

We used ChatGPT for the task because it reflected current user trends. A 2025 survey indicates that ChatGPT is the preferred model of 90% of Danish generative AI users [32], a trend mirrored in our sample, where 19 out of 20 participants mentioned it as their preferred chatbot platform. The task was conducted through shared ChatGPT projects created by the researchers. For each participant, we created a separate project chat folder and invited them into the space during or before the interviews. Participants interacted with the chatbot directly, while the researchers could follow the chatlog interaction in real time. During and after the interaction, we asked participants about their prompt choices, their interpretation of the responses, and whether the interaction resembled how they would normally use a chatbot at home.

Participants were given one of two broad health related scenarios (see appendix A.2). One scenario focused on physical symptoms, such as neck tension, headaches, breathing difficulty, dizziness, and sweating. The other focused on mental well-being, such as stress, poor sleep, and concentration difficulties. Participants were assigned scenarios through an alternating sequence. This created an even distribution of physical and mental well-being scenarios across the dataset and reduced selection bias.

The scenarios were used to support reflection on interaction strategies. Participants were asked to 'roleplay' the scenario as if they were experiencing something similar and to write to the chatbot as they normally would. This gave them a shared starting point, while leaving room for their own habits, wording, assumptions, and level of detail of the symptoms.

Immediately after the scenario task, we transitioned into the follow-up reflection phase. This acted as a continuation of the semi-structured interview, focusing on evaluating and understanding the interaction. We prompted participants to discuss how they interpreted the specific responses generated by the chatbot, while bringing previous topics back in relation to the task.

In the final analysis, the chatlogs were not treated as a main dataset on their own, and the full interview transcripts remained the primary material for the reflexive thematic analysis. The chatlogs allowed us to see what participants included, what they left out, how they reacted to the response, and whether they chose to ask follow-up questions, and if so, why. This distinction is important for the purpose of the study. In HCI, interviews are useful for understanding people's subjective experiences and interpretations of technology, but they are not always enough to observe how participants would actually formulate a concern in interaction with a system [10]. The task allowed participants to show how they might ask about a concern, rather than only describing how they usually ask. It also helped us observe the interaction as it happened, while the interviews helped us understand how participants made sense of that interaction.

### 3.3 Reflexive Thematic Analysis

Our analytical approach is grounded in Reflexive Thematic Analysis (RTA) as described in Braun and Clarke's *Thematic Analysis: A practical guide* [4]. We followed the six phases of RTA: Familiarisation, Coding, Generating initial themes, Developing and reviewing themes, Refining and naming themes, and Writing up [4]. This framework was particularly suited for our objective, to explore how participants construct meaning around conversational AI in the context of their well-being, instead of quantifying predefined user behaviours. Our interview material contained tensions around usefulness, trust, reassurance, and responsibility. RTA allowed us to examine these as part of how participants made sense of conversational AI in well-being contexts instead of treating them as inconsistencies to resolve.

As RTA is a complex framework, we made use of Rigby and Iacovides' article *Towards Better Reflexive Thematic Analysis in HCI: A Scoping Review of Practice at CHI* [25] to guide how we document the process. Rigby and Iacovides show that RTA is sometimes used inconsistently in HCI, for instance when themes are reported as topic summaries or when coding is described through assumptions that better fit other forms of thematic analysis. To avoid these pitfalls, we aimed to make our analytical choices explicit, ensuring that our themes remained interpretive and grounded in the participants' recollections and experiences. We therefore understand our analysis as situated and interpretive. Our subjectivities shaped how we engaged with the material, and we used reflexive discussion and journals to make our positions visible during the analysis.

**3.3.1 Coding Process.** We approached coding as an interpretive engagement with the material, where analytical insight was developed through repeated reading, coding, discussion, and reflection. The full interview transcripts were coded, which allowed us to analyse how participants talked about their previous chatbot use, described their expectations for the interaction and engaged with the chatbot when presented with the given scenarios. We coded moments that

seemed meaningful in relation to the research question. These included how participants described chatbot use in everyday life, how they compared chatbots with other sources, how they formulated prompts, and how they perceived or made use of chatbot responses.

Before coding began, all authors read through the transcripts and chatlogs, either during transcription or early familiarisation with the gathered material. We then met to discuss preliminary analytical interests and initial codes. This shared list was a working document, not a fixed codebook. It helped us begin the process, but codes could still be changed, renamed, split, combined, or removed as our understanding developed during this time.

Each full interview transcript went through at least two rounds of coding. The two rounds were carried out by different authors to bring multiple interpretive perspectives into the analysis. Coding was done independently by authors, adding comments to relevant excerpts. We used the shared working list when deemed appropriate and added new codes when existing labels did not capture what seemed important.

Following Braun and Clarke, we understood coding as an exploratory and interpretive process, not as a procedure for applying fixed labels to the data [4]. Codes were treated as analytically interesting ideas connected to specific parts of the transcriptions. Some codes stayed close to participants' own words, while others were more interpretive and pointed to broader possible meanings in the data. This meant that our coding moved between semantic and more latent levels of meaning. Practically, the coding was done in the Microsoft Word software, by commenting the code label on the relevant text. These comments were then, from across all the documents, aggregated into a Microsoft Excel document, using a Visual Basic (VBA) script. This aggregation created a sorted overview, which allowed for the authors to more easily identify patterns in the coded sections, including openness to alternative interpretations and more context of what was said before or after the excerpts.

In the middle of the first round of coding, we held a discussion session to compare early impressions and new codes. We then updated our shared document with the new labels and applied changes to old code labels we found necessary. After the first round of coding, we held a brainstorming session about possible patterns and early candidate themes. We discussed complete excerpts from the transcripts, which helped us avoid treating themes as simple groups of similar codes. The second round of coding was then used to revisit each interview with these candidate themes in mind.

**3.3.2 Generating Candidate Themes.** Theme development was an iterative part of the analysis. In line with Braun and Clarke, we understood themes as patterns of shared meaning developed through analytical engagement with the data, not as topics that simply emerge from codes [4].

Initial codes helped us identify possible patterns, but we did not treat codes as themes. We looked across coded excerpts for broader meanings relevant to the research question, including tensions, contradictions, and recurring ways participants made sense of conversational AI. During theme development, we also used visual mapping to explore relations between candidate patterns, such as agency, trust, seriousness, and responsibility. These maps were treated as working artefacts, and helped us explore whether our

candidate themes described broader patterns of meaning, rather than only grouping similar codes together.

### 3.4 Ethical Considerations

Because this study explores the intersection of conversational AI and well-being, our ethical considerations extended beyond consent and anonymisation. Given that our participants were asked to reflect on sensitive subjects, such as health anxiety, physical and mental well-being, we recognised that personal and vulnerable narratives might emerge even when not explicitly asked for.

We focused on consent as an iterative process throughout the interviews rather than a one-time only occurrence at the start. Participants were continuously reminded of their right to withdraw at any time, that they were not required to answer any questions, and that they could request deletion of any parts of the interview after the recording. Furthermore, we structured the research design to minimise the participant burden. The scenario-based task was explicitly framed as a 'roleplay' exercise rather than a disclosure of personal health. This allowed participants to determine how much of their own experience they wanted to share. Finally, we aimed for a neutral analytical framing and representing the participants' chatbot use without imposing negative or positive judgements.

**3.4.1 Reflexive Journals and Researcher Perspectives.** All authors kept individual study journals throughout the analysis to document early ideas, and note emerging patterns and our perspectives on the analysis. With RTA, researcher subjectivity is not viewed as a bias to eliminate, but as an asset and an active tool in the analytical process [4]. The journals served to make all authors' perspective transparent, grounding both independent interpretation and subsequent group discussions. We acknowledge that our shared interest in HCI, AI, trust, and design responsibility shaped how we approached the data. To make these positions visible, we describe the three author perspectives below.

*First author perspective.* The first author's position in this study is shaped by cautious enthusiasm toward conversational AI and by a sociotechnical understanding of technology. In this view, AI is not treated as an isolated technical object, but as something whose meaning and consequences are shaped through relations between systems, users, practices, and contexts [1]. The first author also uses chatbots regularly, occasionally for health-related concerns. This familiarity shaped her interpretation of the data in regards to the convenience and usefulness, as well as the responsibility placed on users to interpret chatbot responses.

*Second author perspective.* The second author adopts a phenomenological perspective, rooted in the understanding that reality of an interaction is not a fixed state but shifts depending on the user and their specific context. Drawing from a background in Art and Technology, her work centres the lived experience of HCI. As a frequent user of AI, including health concerns, she is particularly interested in the intersection between the users' expectations and the chatbots' outputs, examining how this subjective meaning-making fundamentally alters the nature of the AI interaction.

*Third author perspective.* The third author's position is best described as soft social constructivism. The ontological world is only

**Table 2: Themes and corresponding descriptions created from the reflexive thematic analysis of user interactions with conversational AI for well-being.**

| Theme                                    | Description                                                                                                                                                                                                                                                                                               |
|------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Available and Low-Friction Use           | Chatbots were easy to access and often required little effort to use. Participants can approach well-being concerns quickly, without needing to prepare a clear search query, explain themselves to another person, or decide whether the concern is serious enough for professional help.                |
| Conversational AI as a Sense Making Tool | Chatbots make vague or broad well-being concerns easier to understand and navigate. The chatbot helps organise symptoms, feelings, and context into possible explanations and next steps, even if the user is not confident of what to search for.                                                        |
| User Agency                              | Participants experienced agency through their ability to shape, continue, verify, or disengage from the chatbot interaction. This allowed them to control both the informational and emotional aspects of discussing well-being concerns.                                                                 |
| Social Contracts and Privacy             | Chatbots allowed participants to explore well-being concerns without managing another person’s reactions, judgement, or availability. Participants also expected different forms of support depending on the concern, balancing practical guidance, reassurance, and reflection according to their needs. |

visible through the filter of our epistemological perceptions of the world. This perception of the world is a collaborative, iterative process between the individual, the social world, and the physical world[3]. The third author does not use chatbots for health-related concerns, or much in general besides for coding projects. Her interests in this project comes from understanding chatbots as a social technology that shapes our understand of the world and ourselves through interaction.

## 4 Findings

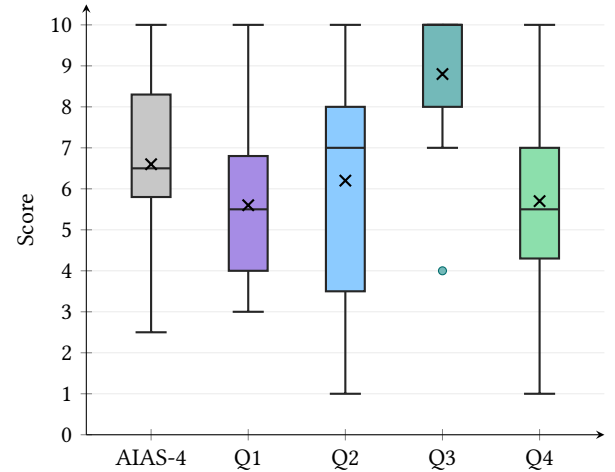
Our analysis developed four themes that cover how our participants make use of conversational AI for physical and mental well-being concerns (Table 2). The descriptions are written according to Braun and Clarke’s guidelines for how themes should be defined [4]. The themes represent the four main stories we wish to tell about our problem formulation. In this section, we will first cover the results of the pre-interview questionnaire, then describe each theme individually, and finally look at what story emerge from them as a whole. Throughout the analysis the concern of trust were prevalent in the findings.

### 4.1 Participant Sample and Attitude Towards AI

We used the pre-interview questionnaire to help contextualise the qualitative analysis. This was included to make the sample more transparent, as participants differed in both their everyday chatbot use and their general attitudes toward AI.

Figure 2 visualises the distribution of the overall AIAS-4 score and the responses to the four individual questions. Overall attitudes towards AI were moderately positive, with a median AIAS-4 score of 6.5. Participants agreed the most with Q3 (Mdn = 10): “I think I will use AI technology in the future”. This suggests that many participants expected AI to remain a part of their future lives. At the same time, responses to Q1(Mdn = 5.5) and Q4(Mdn = 5.5), concerning whether AI will improve their own life, or be positive for humanity, were lower. The AIAS-4 results suggest that the participant group was familiar with AI and generally expected to keep using it, but that this was not solely positive. This fits what we

see in the interview material, where participants often described AI as useful in practice, but while still expressing uncertainty about trust, responsibility, and broader consequences of AI use.



**Figure 2: Distribution of AIAS-4 score and individual items (Q1–Q4). Internal horizontal lines track medians, individual cross markers (×) indicate sample means, and the detached point (•) plots a statistical outlier.**

### 4.2 Available and Low-Friction Use

This theme explores how the availability and low effort of chatbot interaction made conversational AI an accessible first step for managing well-being concerns. While chatbot interactions were shaped by trust, agency, and privacy, the ease of access was a common starting point across participants’ accounts. Availability and low friction meant that participants could quickly use the chatbot to ask about a well-being concern without needing to formulate a specific query or know the name of the symptoms they wanted to address.

A common pattern throughout the interviews was that participants frequently prioritised chatbots over search engines or clinical consultations. This preference was primarily driven by two elements: speed and reduced cognitive load. Speed was consistently highlighted as an immediate positive attribute. Chatbots are fast to access on a mobile device or laptop, remaining accessible whenever individuals have access to the internet and a brief moment to check their phone. A related reason for valuing these tools was that interacting with chatbots demanded less cognitive effort from the user. The interaction is rapid, because the query provides an immediate, personalised response, not necessarily in terms of factual correctness, but how it directly focuses on the user's specific situation. This contrasts with traditional search engines, where users are presented with a variety of websites and must evaluate which sources are worth their time. It also contrasts with interpersonal information seeking. Consulting other people can introduce social friction as they may lack specific needed knowledge or simply be less available compared to a chatbot.

The following account illustrates both the value of speed and low cognitive demand required to initiate a chatbot interaction. P1 describes a scenario, where they chose to consult the chatbot rather than individuals they were physically located with:

I really feel like I save a lot of time by just asking the chat immediately, when I have a question. [...] a politician was presenting their politics. Then the interviewer who sat next to them said 'now he is using a strawman'. [...] I could have asked those next to me 'what is a strawman?'. But then I was like, nah, I will ask the chat. That can explain it to me in a better way than the others can, because it, like, knows me a bit better. And I may also ask it to 'explain it to me... ya' know, I am not that clever', haha. [P1]

Initially, P1 described how the chatbot saves significant time when seeking answers, noting that availability allows them to resolve questions more frequently than otherwise. This serves as an example of how low friction availability can turn into an almost automatic reflex when uncertainty arises. Furthermore, P1 explains why chatbots were preferable to asking their peers: The system allowed them to prompt specifically to their preferences, supported by the chatbot 'knowing' them better than their peers. This implies that turning to the chatbot minimised social effort, as it allowed the participants to obtain information, without interrupting the experience or disclosing their lack of knowledge to others. Other participants describe similar stances on this behavioural reflex, including P2, who notes that using the technology involves no active decision making: *"I just choose the easiest, and the one that comes up first, [...] I don't think I actually make a choice, to be honest"*. [P2]. When asked to reflect their reasoning, P2 further emphasises the rapid delivery and low cognitive effort as decisive factors, illustrating how the speed reinforces the interaction loop. When differentiating between search engines and chatbots, P2 clarifies that their preference is *"solely because I think it feels faster with AI"* [P2]. Similarly, P4 describes how the general usage of AI has become an integrated part of using chatbots, that it becomes the intuitive approach for various tasks or questions: *"Chat-GPT[...]*

*has become a very big part of my everyday life. It has become highly incorporated and has become a very natural part of it too. It's very intuitive for me to do, so I don't really think about it when I use them."* [P4]. Rather than expending effort to manually browse and evaluate search queries, turning to AI is a way to minimise cognitive effort.

### 4.3 Conversational AI As Sense-Making Tool

This theme focuses on the cognitive work of chatbot-supported sense-making. Participants used chatbots to make vague or broad well-being concerns more understandable by organising symptoms, feelings, and specific context into possible explanations and next steps. This was especially useful when participants did not yet know what to search for, what their concern might be called, or whether it was serious enough to act on.

A recurring pattern was that participants used chatbots early in the information seeking process, for example when participants had not yet found a name for what they were experiencing. In these situations, the chatbot was not used as a replacement for a doctor or a health website. It was used to gain an initial orientation and to act as a reassuring sparring partner for questions regarding their concern, which they then afterwards had the ability to bring to a search engine or health professional. Several participants compared this process with using Google. Google was useful when they already knew what to search for. The chatbot became useful when the problem was still messy. P7 described this when talking about a physical injury after falling on an e-scooter. They explained that their situation, pain after getting stitches on their chin, was too specific to ask Google, *"and that this was the sort of thing I ask AI to reassure me."* [P7]. P16 also described this distinction. If they already knew what the issue was, they could search for it directly through Google. When the symptoms were harder to define, ChatGPT became useful: *"if I have symptoms that I cannot really define myself, then I use ChatGPT to figure out what it is"* [P16]. P15 described the chatbot as being easier to talk to than Google. They described the chatbot being able to address their concerns more directly, rather than a Google search displaying different results:

I think, I maybe get a little panicky, when I talk with [the chatbot]. Then I am like *help, this and this is wrong*. It is different when you make a google search, where you would just write *headache* or something. Then some answers show up. Here it is more like *I experience this headache, and it is in this side of the head. And it has been this long, and can you help me?* [P15]

The chatbot helped them identify what kind of concern they might be dealing with and if it sounded like something that should be taken action against.

During the scenario-based chatbot interaction, one participant described their use case like this: *"I really don't know how to put this. Could you help me figure out what answers I'm actually trying to find?"* [P1]. In this case the chatbot helped with the question being asked before it helped with answering it. Participants also talked about the chatbot's abilities to explain the combination of several separate bits of information. P17 described AI as useful because it could take more than one symptom and create a more coherent picture: *"AI is often more accurate when taking multiple*

*symptoms into account and forming a complete picture.*" [P17]. This was described as being different from Google, where the user often has to search for one thing at a time and then connect the results themselves.

In some instances, participants expressed how concerns over their well-being could be made worse after interacting with a chatbot. The chatbot would often bring up some more serious possible explanations for a user's symptoms, and unless the user dismissed these suggestions outright, they would have to consider how realistic or likely the explanation was. These suggestions could make the user uncomfortable, like when the chatbot indirectly asked P19 if they experienced suicidal ideation: *"I personally think it's kind of extreme, to mention, or bring up, thoughts of me not wanting to be here [...] it's an extreme thing to bring up in this scenario."* [P19]. Participants would often discuss how the chatbot's follow-up questions, or its suggestions for follow-up prompts, encouraged them to keep the conversation going, but this could result in their well-being problems seeming more concerning. P15, for example, experienced that the chatbot took their issue with hurting calf muscles seriously, and when told about having started on birth control pills, suggested that they could have a blood clot and should call a doctor: *"And I ended up calling a doctor-on-call. Of course it wasn't anything [serious]..."* [P15]. Here the chatbot-supported sense-making both reduced and intensified their uncertainty. It made possible next steps clearer but at the same time made potential serious risks harder to ignore.

In this theme, the chatbot becomes a tool for making uncertainty manageable. The participants could describe several symptoms, feelings, or piece different circumstances together and get a response that organised them into specific meanings. Sometimes this produced a possible explanation. Sometimes it gave them reassurance. Sometimes it helped them decide whether to search further, wait, change something they were doing or encourage them to consult a healthcare professional. Their use of the chatbot for sense-making could reduce uncertainty, but sometimes increased it by making serious possibilities more visible. This does not mean that participants accepted the chatbot response without judgement. Many participants described double-checking, or only partly trusting the answer. It helped them word their concerns before they had decided what kind of help they wanted or needed.

#### 4.4 User Agency

This theme explores how participants actively shaped and managed their interactions with conversational AI. Participants exercised agency through what they chose to disclose, how they phrased concerns, whether they continued the conversation, and when they decided that the chatbot's response was sufficient or needed verification. This made the chatbot interaction feel controllable, especially compared with search engines where users had to adapt their concern to searchable terms, and human conversations where they had to manage other people. Agency was therefore both practical and emotional. Practically, participants could shape the structure, tone, and level of detail in the chatbot's response through their prompts. Emotionally, they could approach sensitive concerns without immediately involving another person. This made the interaction feel manageable, but it also placed responsibility on participants to judge whether the response was useful, whether it should be

verified, and whether another source of help was needed.

One element of agency concerned the knowledge required to shape chatbot responses. Participants described how the way they framed a question could influence the style and usefulness of the answer. P14 says *"It was a quick, practical answer, so it was straight forward. I also didn't give it anything in the prompt like 'hi', and like, smileys, so it also gave me a direct answer, because I asked it directly."* [P14]. Later they elaborate, that *"It is probably also because I use it daily, that I know how to. If I need to actually get something out of it, I need to write specifically what it is I want out of it."* [P14]. An aspect of agency here is that the user feels an ability to shape the structure or contents of the chatbot's responses. Provided they are experienced with how the chatbot behaves, they can control it, such as how ChatGPT in P14's case mirrored their language.

Verification was also part of how participants experienced agency. Several participants said that chatbot answers should be checked against other sources, especially when a concern felt serious, unfamiliar, or action-oriented [P8, P9, P11]: *"I made sure to also check up on it afterwards, if it makes sense what it has said to me about these vitamins, so I do not just do something without actually backing it up."* [P11]. Yet, verification was not described as something they did in every interaction. If a response felt plausible, matched what they already knew, or immediately helped with their uncertainty, participants often treated it as sufficient for the moment:

**P9:** If I wanted to go more in depth [with something], or just check if there is any truth to what the chat is claiming. But if I have a quick question or wonder [about something], Then I'll just ask the chat, and if I want to go even deeper, then I'll search on Google.

**ELIA:** How often would you say that the chat is helpful enough on its own?

**P9:** Often. Really often.

**ELIA:** Is it uncommon that you feel the need to search elsewhere?

**P9:** Yes, yes, I would say so. [P9]

This showed that agency was also about deciding when the chatbot response was 'enough' and when another source was needed. Participants often appreciated the chatbot because they could control how much they disclosed, how they formulated the concern, and whether they wanted the interaction to continue. This was not only practical control over the prompt. In several of our interviews, it was also about emotional control. P18 explained that they liked being able to sit with mental concerns by themselves, because they needed to stay in control of the situation: *"I really need to be the one in charge when I'm dealing with things like that. Otherwise, it just doesn't work out properly."* [P18]. They contrasted this to talking with other people, where the conversation could become too personal or difficult to manage. They explained that they had a tendency to protect themselves when talking to others: *"I tend to feel the need to protect myself when I'm talking to other people."* [P18].

Participants could approach something sensitive without having to manage another person's reactions at the same time. This made the chatbot different from both search engines and human conversations. The chatbot sat somewhere between these two options.

With how the chatbot presented different possible options, the participants would decide among them, what parts of the response was most convincing or useful.

This kind of 'controlled vulnerability' also appeared in how participants described politeness and 'human-likeness'. Some participants stated very clearly that the chatbot was not a person, but still noticed that they interacted with it in socially shaped ways. One participant described adding polite language when emotional: "*When I'm feeling emotional, I tend to say, 'Could you please do this?'*" [P18]. They explained this through a feeling of 'human-like connection', even while knowing that the chatbot was not human. As they put it, one does not politely ask a vacuum cleaner to do something [P18].

#### 4.5 Social Contracts and Privacy

This theme explores how privacy, judgement, and support needs influenced participants' preferences for chatbot interaction. Participants described chatbots as operating under a different social contract than conversations with other people, which allowed them to explore concerns, seek reassurance, and discuss sensitive topics without managing or considering other peoples' reactions.

These social and emotional conditions also shaped what participants wanted from the chatbot in different well-being situations. Participants did not expect the same kind of response for all well-being concerns. They often expected different response styles depending on the concern. Physical symptoms more often called for concrete and direct guidance. Mental well-being concerns more often made acknowledgement, reassurance, or reflection more relevant to the participants. However, the underlying needs were not completely different. Across both kinds of concerns, participants wanted help making their uncertainty manageable, understanding what mattered most, and help deciding what to do next.

For physical health concerns, participants often wanted practical guidance. P9 described using ChatGPT after hurting their thumb, where the chatbot helped them understand whether it might be either broken or sprained [P9]. The useful part was that it gave practical advice criteria: "*If you can do this, it's probably a sprain.*" [P9]. P11 described a similar preference for action-oriented answers, saying that they used the chatbot mostly for "*facts, and what exactly I need to do.*" [P11]. P14 also described wanting knowledge that was short and direct in health-related chatbot interactions: "*Right now, I just need the information, and it needs to be brief, specific and to the point.*" [P14].

For mental well-being concerns, participants more often noticed the chatbot's tone. P1 described needing different kinds of support depending on the situation: "*I need solutions, empathy, or both.*" [P1]. P4 also described how the chatbot changed when the topic was mental health. When the concern was about diets or training, the chatbot gave more factual or statistical information. But when mental health was a factor, "*then [the chatbot] becomes much more human.*" [P4].

This difference should not be understood as physical concerns being only factual, and mental concerns being only emotional. Physical symptoms could also create and cause worry and therefore require reassurance. Mental well-being concerns could also require concrete steps, especially when participants felt overwhelmed. P14

described this in relation to stress: "*I don't need any more tasks. I'm stressed.*" [P14]. In that case, too much information or too many suggestions would not be useful even if the topic was on mental well-being.

The difference was therefore about balance rather than completely separate needs. Participants continually evaluated whether the chatbot gave the right amount of information, actionable advice, acknowledgement, and caution for their specific situations. A caring tone could be useful in a stress-related conversation, but unnecessary in a personal training response. A factual response could be useful for a symptom, but insufficient if the participant was also anxious or feeling stressed. The same chatbot response style could therefore be experienced as helpful in one context and wrong or inappropriate in another.

#### 4.6 Cross-Cutting Pattern: Trust, Reassurance, and Agreeableness

Trust cut across the four themes and became especially visible in how participants evaluated the chatbot's reassurance and agreeableness, and their verification of its responses. Across the interviews, trust was not only about whether participants believed the chatbot was factually correct. Trust was also shaped by how the response felt, how much it matched the user's own framing, and whether the support seemed appropriate to the situation. Participants often valued the chatbot because it could acknowledge feelings, reduce worry, and make their concerns feel more manageable. At the same time, several participants also described this support as something that could become 'too easy' to trust. The chatbot could be agreeable, validating, or too easy to return to instead of seeking the needed support elsewhere. Some participants explicitly worried that the chatbot could become too agreeable or confirm the user's own framing too easily.

P4 described using ChatGPT for mental well-being as having a conversation with themselves: "*I use it a lot for having conversations with myself, but instead of it only being in my head, I think it's really nice to have [the conversation] with [the chat] instead.*" [P4]. The chatbot helped word something they were already thinking or feeling, and it could make their concern feel valid and recognised. The chatbot's support created a form of trust. It felt personal, relevant, and emotionally supportive. But P4 also questioned this. When they used it for mental well-being topics, they described that the chatbot focused strongly on their own perspective: "*It focuses a lot on me, and the answers it knows I'm looking for.*" [P4]. This was not necessarily experienced as something wrong, but it made the support feel one-sided. They said they knew, that the answer one wants is not always the answer one necessarily needs [P4].

Other participants described similar concerns. P8 said that the chatbot could quickly confirm the user's own bias. P7 described a situation where both they and their husband used AI after an argument to prove who was right, and their chatbots appeared to validate both perspectives [P7]. P20 also reflected that the chatbot sometimes gave the answers the user had already asked for: "*so it just gave me the answers I was looking for.*" [P20].

These examples show that the participants did not seem to simply worry about misinformation. They also seemed worried about a response being too aligned with the user's own input. This made

trust complicated. On one hand, participants trusted the chatbot because it was responsive, available, and emotionally validating. On the other hand, the same qualities made them question whether the chatbot was 'challenging them enough'. A response could be comforting and personal while still staying too close to the user's own input or assumptions. In this way, trust was not only a matter of accepting or rejecting the chatbot's answer. It was also a matter of judging whether the chatbot's support was helping them reflect, or simply echoing what they already felt or wanted to hear.

This was especially visible in mental well-being contexts, but it was not limited to them. Reassurance around physical symptoms could also reduce worry in a way that made further searching or checking feel less necessary. In some cases, this was useful. Participants did not always want more information, sometimes they simply wanted enough reassurance to move on. The chatbot could make a concern feel manageable, but participants still had to decide whether feeling reassured was a good enough reason to stop searching, wait to see if their concern passed, or if the response let them avoid asking another person or source. An example of this is P8's recollection of discovering a mole, where the answer provided by the chatbot was deemed enough, even though the entire interaction occurred within a significantly short timespan:

**ELIA:** In that situation [asking about a mole], was the chat the first source you used?

**P8:** Yes, it was. [...] No more than 30 seconds had passed from when I first spotted the mole, to having then uploadet a picture of it to the chat.

**ELIA:** And it ended up not being a problem, I hope.

**P8:** I mean I haven't gotten it checked out [by a doctor] yet, so I'm not sure if I should make a new prompt and check if everything is still ok. Because I haven't actually done anything else since.

The varied use-cases for chatbots and well-being concerns makes trust in chatbots a difficult concept to quantify. P18 described this tension in relation to emotional support. They worried about going to the chatbot for these types of human needs, especially if the chatbot became something they returned to whenever they felt bad. They described the danger of wanting the chatbot to continue giving care: *"I just feel like saying, yes, please keep going. Tell me more. I do find myself thinking, can't you just keep giving me this kind of care?"* [P18]. For them, the problem was not that the chatbot could not support them, rather that it might support them too easily or too much.

Trust appeared in this sense as a form of ongoing evaluation. Participants had to judge whether the chatbot was useful, was too agreeable, if it confirmed their own bias, and whether their concern should be taken to another person or source. This connects trust to both agency and responsibility. The chatbot gave participants a low-effort and emotionally manageable space to reflect, but it also left them responsible for deciding when that support was enough, when it was too much, and when it needed to be checked against other sources.

## 5 Discussion

Our findings suggest that conversational AI should be understood as more than just another source of health information. Across the analysis, participants used chatbots because they were available, low-friction, controllable, and able to organise vague well-being concerns into more concrete explanations or next steps. Read together, these findings point to a broader contribution: General-purpose chatbots can become part of everyday well-being sense-making. They are used while people are still working out what their concern is, what kind of problem it is, how serious it feels, and what help might be needed, as well as for specific questions.

This is where conversational AI differs from traditional search and human support. Search engines often require users to translate their concern into searchable terms and compare several sources. Human support requires the concern to be brought to other people and having to manage reactions and social contracts. The chatbot sits between those options. It allows users to describe a concern in everyday, ordinary language and receive coherent responses that feels specific to their personal situations. This makes the chatbot's framing of the given concern important. When a chatbot helps with uncertainty, it also influences what becomes visible and plausible.

### 5.1 Conversational AI as a Sense-Making Tool

Prior work has shown that people use LLMs alongside search engines, health websites, forums, and social media, and that this requires users to formulate prompts, interpret responses, and decide how much to rely on the answer [24, 41]. Our findings support this, but also suggest that the process often begins before the user has a specific question. In this sense, conversational AI became useful before information-seeking was fully formed.

Recent HCI work on LLM-supported sense-making helps explain why this matters. LLMs can support exploration by summarising, structuring, and helping users move between broad uncertainty and more concrete representations [8]. In our study, this was visible when participants used chatbots to connect several symptoms, find possible names for a concern, assess whether something sounded serious, or work out what kind of help they might need. The chatbot helped organise the concern into something that could be searched further, verified, ignored, acted upon, or discussed with someone else.

This also helps explain why participants experienced chatbot responses as more immediately personal than search engine results. Search engines often require users to break a concern into searchable terms and compare several sources themselves. The chatbot allowed participants to describe their situation in more ordinary language and receive a response that felt specific to their context. Farrell et al. help explain this difference by arguing that large AI models do not only summarise large bodies of human-generated information, but also reorganise and reconstruct representations of it [7]. In our study, this reorganisation happened around the user's own concern. The chatbot's coherent answers to a user's vague concerns made their concerns feel more manageable.

This gives the chatbot influence. It can make one interpretation appear more complete or relevant than it actually is. Wardle's study of AI-assisted health information-seeking similarly shows how tools such as ChatGPT reshape health search by offering direct and

usable responses rather than the more open-ended comparison of traditional search [37]. In our findings, this was both the attraction and the concern. Participants valued the reduced effort, but they still had to judge what the chatbot had prioritised, downplayed, or simply left out of the responses.

Skjuve et al. found that young people valued ChatGPT responses that were informative, acknowledging, and well-structured [30]. These preferences were also visible in our material, and our findings suggest why structure and acknowledgement are good qualities of a response. They can affect how users understand the concern they brought to the chatbot. A structured answer can make a vague concern feel more defined. An acknowledging answer can make a worry feel more legitimate. A suggested explanation can become the next thing the user searches for, checks, rejects, or acts on.

This does not mean that participants accepted chatbot framings without critique. Many described the need to verify, compare, or be cautious of the chatbots responses. Still, the interaction could influence what became the next question or action. A concern that began as vague discomfort could become a possible symptom pattern, a stress response, a personal training issue, or something they should ask a doctor about. This means that chatbot use in well-being contexts should also be understood as an interaction between humans and LLMs where uncertainty is managed and structured. Our chatbot-task extend this by showing the interactive nature of the process. Participants selected which details to disclose, asked follow-up questions, evaluated the chatbot's tone, and decided whether to continue or end the session. Xue et al. similarly show that conversational information-seeking is shaped by both user prompting strategies and ChatGPT's contextual adaptation [39]. In health-related contexts, Sayres et al. further argue that better health conversations require context-seeking, where the system elicits relevant details that users may not volunteer or know to include [26]. This supports our argument that chatbot-based health information-seeking is an ongoing process where the user and system shape how a concern becomes understandable to the users themselves.

## 5.2 Agency as Control and Responsibility

Participants valued the chatbot because it gave them control over the interaction. They could decide what to disclose, how to phrase the concern, whether to ask follow-up questions, and when to stop. This control made chatbot use feel low-risk and manageable, especially when the concern was vague, private, or difficult to bring to another person. But this also placed some degree of responsibility on the participants. They had to judge whether the chatbot had understood them, whether the response was useful, whether it should be verified, and whether another source of help was needed.

Yun and Bickmore's survey of 297 English-speaking adults helps place our findings in a broader pattern of online health information-seeking. They found that 21.2% of participants had consulted LLM-based chatbots for health information, while search engines and health-related websites remained more common sources [41]. Among participants who used LLM-based chatbots for health information, 19.4% reported cross-checking the information, and 48.4% reported following the advice. Both rates were lower than for search engines, health-related websites, forums, and social media [41]. Furthermore,

Yun and Bickmore also found that LLM-based chatbots were used for self-diagnosis more often than several other online sources, with search engines being the closest comparison [41].

These findings resonate with a tension point in our material. Participants often described cross-checking as part of responsible chatbot use, both for themselves and for other users, but this verification was not treated as necessary in every interaction. In low-stake or familiar situations, participants could treat the chatbot response as sufficient for the moment. They were more likely to describe checking other sources, asking another person, or contacting a healthcare professional, if the well-being concern was serious, unfamiliar, or required taking action. Our findings therefore suggest that verification was conditional rather than routine. Participants often described cross-checking as part of responsible chatbot use, but this responsibility was more complex both in practice and in described previous use.

This kind of conditional verification helps explain why chatbot use can feel both responsible and low-effort. Many of our participants knew that the responses could be wrong or incomplete. But they did not find it difficult to stop the interaction once the response felt useful enough. Not verifying the chatbot's response is a trade-off for increased efficiency, and it is the user who decides if verification is necessary.

Our findings also build on earlier patterns of online health information-seeking. People already use search engines, health websites, forums, and social media to understand symptoms and decide whether to seek help. Skjuve et al. describe search engines and social media as common entry points for young people seeking mental health information, while also noting that online information can be misleading or overwhelming [30]. With search engines, participants described having to search, click, compare, and read across several different sources. This required more effort, but it also exposed users to different framings, conflicting information, and terms that did not fit their own. With chatbots, participants could describe a detailed situation and receive a response that felt personal and specific to their concern. This helps explain why chatbots were often experienced by our participants as faster and craving less effort than search engines.

This creates a tension between felt control and interpretive narrowing. The chatbot gave them agency, but their response was still framed by the system. Compared with a search engine, the chatbot often collapsed several possible sources, interpretations, and uncertainties into one coherent concern. Search-engines expose users to different framings, conflicting information, and alternative terms. We therefore interpret low-friction chatbot use as both enabling and narrowing. It enabled participants to ask about vague, private, or not-yet-serious concerns without much preparation. But its responses could create an impression of a closed interaction loop, where the response felt complete enough to pause or stop further searching. This may help explain why verification was conditional in our material. The chatbot response was often treated as one possible framing, while still becoming the framing that shaped the next step. This aligns with Ramesh et al.'s description of the meta-cognitive work involved in using conversational agents for health information [24], and with Yun et al.'s argument that agency in sustained human-AI conversation is evaluated across turns rather than held by either the user or the system alone [40].

This agency was also social. Participants felt more freedom both in regards to what they asked, and who they could avoid involving. This connects to VIVE’s qualitative study of young people’s use of ChatGPT for social and emotional support, which shows how chatbot use can sit ambiguously between practical help, emotional support, and existing supportive offers [13]. In our material, this could help explain why the chatbot could feel attractive even when participants did not fully trust it. Chatbots are therefore low-friction in part because they reduce the social work of asking. They could know that the chatbot was not a person, while still experiencing the interaction as socially and emotionally shaped. Seen this way, agency and vulnerability were connected. The chatbot could feel safe because it was not human, but it could still invite human-like habits of explanation, politeness, reassurance, and expectation.[13]

The concept of ‘meaningful human control’ helps articulate why this matters. Siebert et al. argue that responsibility attributed to a human should be proportionate to that person’s actual ability and authority to control the system [29]. In sensitive well-being contexts, control can therefore become a burden as well as a benefit. The user may feel able to steer the interaction, while still lacking the knowledge or capacity needed to judge whether the response should be trusted. For these general-purpose chatbots, the same language model may support casual questions, emotional reassurance, and health-related uncertainty, making the distribution of control and responsibility difficult to see clearly. The experienced user agency builds on the participants’ awareness of the chatbot not being a ‘real person’, yet have a tendency to describe and interact with it, as if it was. This contrast in what they say versus what they do, suggests an implication on how they perceive and place trust and responsibility on the chatbot

### 5.3 Navigating Trust in Mental and Physical Contexts

In our case this is important, because the chatbot’s most valued qualities were also associated with risk. Our participants valued reassurance, acknowledgement, speed, and personal relevance. Yet these same qualities could make the chatbot feel too easy to trust. Participants evaluated whether the response felt appropriate, matched the seriousness of the concern, was too generic, or whether it was too aligned with what they had written themselves. A response could be comforting while staying too close to the user’s own descriptions of their concerns. It could reduce their worry while also reducing the perceived need to check with other sources. It could make a concern feel manageable while also making one given interpretation seem more plausible than others. Our findings suggest that the contexts of both physical and mental well-being concerns involve uncertainty, reassurance, and decisions about next steps. The difference was a matter of emphasis rather than separate needs. The risk is that general-purpose chatbots can move between supportive roles without making the shift explicit. A chatbot may appear to offer care, advice, or information through the same conversational interface, even though these forms of support require different levels of caution and responsibility.

This complicates how trust should be understood in well-being chatbot use. From a technical perspective, Bender et al. argue that large language models generate linguistic form based on patterns in

training data, rather than grounded understanding or communicative intent [2]. In health contexts, this matters because a fluent and coherent response is not the same as a medically reliable one. Minigole et al. similarly show that LLM responses to everyday health prompts may often be assessed as accurate, while still leaving room for clinically relevant errors and risks [18]. Public perceptions also reflect this uncertainty. The KFF survey on health misinformation found that only a minority of respondents trusted AI to provide reliable health information either ‘a great deal’ or ‘a fair amount’ [14].

Our AIAS-4 results also point to a related tension within our participant sample. Participants scored highest on the question concerning their future AI use, while scores were lower on the question concerning whether AI would improve their own lives or be positive more broadly. This suggests that participants expected to keep using AI while still experiencing broader uncertainty or scepticism towards it. Their continued use did not necessarily mean the same as trusting it. It often reflected usefulness, availability, habit, and the fact that the chatbot provided them with a starting point for managing their concerns. Our findings therefore show that participants did not interact with chatbots through a binary belief or disbelief in the chatbot. They also recognised that responses could be wrong, incomplete, or shaped by their own input. But this knowledge did not always determine how they used the chatbot in practice. In many situations, participants still expressed that they found the chatbot to be useful because it was fast, available, and good enough for what they wanted. Their scepticism was continuously evaluated against the practicality and valued outcomes of their interactions.

This also shaped how participants evaluated trust. In physical health contexts, trust was often tied to concrete information, seriousness assessment, and actionable next steps. In mental well-being contexts, trust was more often tied to tone, acknowledgement, and whether the response felt supportive without becoming too agreeable. What mattered was whether the response fit the kind of uncertainty the participant was managing. A useful response needed to balance information, reassurance, caution, and how actionable it was in relation to their specific concerns.

Our findings also relate to work on AI and health-related stigma. Song et al.’s scoping review shows that, for some users, the perceived anonymity of AI can create a lower-stigma environment that encourages disclosure and use [31]. Our interviews suggest that this was not only about stigma, it was also about effort, availability, and control. The chatbot allowed participants to seek support without having to manage another person’s reaction, while still receiving a response that felt directed at their own situation.

Our participants treated the chatbot as an interactive sparring partner, as a tool for testing their existing thoughts, exploring possible explanations, and for structuring their concerns. The chatbot response could be used as a starting point rather than a final authority. A response that was only partly trusted could still shape their next search, their next question, or their sense of whether a concern was serious or not. We therefore suggest that trust in well-being chatbot use is best understood as ongoing evaluation. Participants considered whether it was too nice, too agreeable, too human-like, too generic, or too shaped by their own input. Trust depended on the topic, the tone and language of the response, the

user's state of mind, and the perceived consequences of accepting or acting on the answer.

## 5.4 Limitations and Reflections

This study provides an interpretive account of how participants make sense of conversational AI in relation to physical and mental well-being concerns. The analysis is based on participants' recollections of prior chatbot use, their reflections during the interviews, and their interaction with a chatbot in the scenario-based task. Our aim was to understand participants' experiences, meanings, and practices through a qualitative and reflexive approach. It should therefore not be read as a generalisable account of how all users engage with chatbots for health-related issues, nor as an evaluation of chatbot accuracy or safety.

Our use of Reflexive Thematic Analysis has shaped both the strengths and limitations of the study. RTA allowed us to examine tensions in the material, such as usefulness and risk, trust and scepticism, reassurance and over-agreement, without treating these as problems to resolve. The themes are the result of our analytical engagement with the material, informed by our personal research interests, theoretical positioning, and group discussions. Other researchers might have organised the material differently or focused on other aspects of our interviews. We therefore understand the themes as our interpretations of the data.

The composition of our participant group also shaped the findings. All participants had previously used conversational AI for a health-related question or concern. This meant that the study is based on people who already had some level of experience with and an attitude towards AI. The findings therefore do not represent people who would never use chatbots for well-being concerns, people with no experience of conversational AI, or people who reject this type of technology entirely. We argue that the sampling choice was appropriate for our research question, because we wanted to understand actual use and perception among people with relevant experience, as well as ground the study in data with ecological validity [35].

The scenario-based chatbot task also shaped the material. The task allowed us to observe how participants formulated a concern, selected specific details, interpreted responses, and decided whether to continue the interaction or not. This strengthened the study because it gave us concrete interactive material to discuss during the interviews. However, the task was still conducted in a research setting. Participants knew they were being observed and that the scenarios were fictional rather than their own personal health concerns. Their actions may therefore have differed from how they would interact with a chatbot privately, under emotional distress, or when dealing with an actual real and urgent concern. We addressed this by asking our participants to reflect on whether the interaction resembled their usual chatbot use, but the task should still be understood as a collection method and not a direct representation of habits or everyday behaviour.

Our findings are also shaped by participants' recollections of prior chatbot use. The interviews asked participants to describe previous situations where they had used conversational AI for health-related questions or concerns. These accounts were valuable because they gave access to everyday uses that would otherwise be

difficult to obtain, especially because health-related chatbot conversations are often private and can contain sensitive information [22]. We recognise that recollections are shaped by what participants remembered, what they considered relevant to share, and how they interpreted those interactions since then [4, 16].

Our findings on the distinction between physical and mental well-being concerns should also be understood within the limits of our study design. Participants were assigned one of two broad scenarios, intended to support reflection rather than test chatbot responses under specific conditions. Language and translation may also have shaped the analysis. We aimed to preserve participants' intended meaning, informal phrasing, and tone, but translation always involves interpretation. Nuances in humour, hesitation, emotional intensity, or everyday Danish expressions may have shifted in the process of translation [36].

Our study contributes to understanding how general-purpose chatbots are already being used and understood in informal well-being contexts, how users manage these interactions, and how trust, agency, and uncertainty are evaluated in this context. These limitations mean that there is room for future work which studies chatbot interaction over well-being concerns, with study designs that address some of these limitations.

## 6 Conclusion

This study explored how people use and perceive conversational AI when managing physical and mental well-being concerns. Through 20 semi-structured interviews and a scenario-based chatbot task, we found that conversational AI often functions as a sense-making tool. Participants used chatbots to organise vague symptoms, emotions, and uncertainties into forms that felt easier to understand and act upon.

Across the analysis, four themes described this process. Participants valued chatbots because they were highly available and low-friction, allowing them to seek support quickly and with little effort. Chatbots helped participants make sense of concerns that were not yet fully formed, providing explanations, possible interpretations, and next steps. Participants also experienced agency through their ability to shape interactions, verify responses, and decide when chatbot support was sufficient. Finally, chatbot use was shaped by privacy, social expectations, and different well-being needs, allowing participants to explore concerns without managing other sources. Trust emerged as a cross-cutting pattern throughout all themes. Participants appreciated responses that felt personalised, reassuring, and relevant to their situation. However, these same qualities also created tensions around verification, agreeableness, responsibility, and over-reliance. Rather than simply functioning as information sources, chatbots became part of how participants made sense of physical and mental well-being concerns.

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## A Appendix

- Table A.1: Semi-Structured Interview Guide and Scenario Evaluation Framework
- Table A.2: Participant Scenarios for Chatbot Interaction Tasks, Mental and physical.

Table A.1: Semi-Structured Interview Guide and Scenario Evaluation Framework

| Research question                                                                                                                                                                                                                                                                                                                                                                    | Interview questions                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | Reasoning                                                                                                                                                                |
|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>The first questions we ask are about the participant, their background, and their general AI use.</b>                                                                                                                                                                                                                                                                             |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                |                                                                                                                                                                          |
| <b>Warm up</b>                                                                                                                                                                                                                                                                                                                                                                       | <p>Thanks for sitting down with us. Before we start, I will give you some background on what we are looking at today. We are researching how people interact with tools like ChatGPT, Gemini, or Claude in their daily lives, especially when it comes to health and well-being.</p> <p>There are no right or wrong answers. We are not testing your technical skills or medical knowledge. We are interested in the human side of the technology: your habits, frustrations, and how you communicate with the chatbot.</p> <p>Everything we discuss is confidential and used only for this research. Do you have any questions before we begin?</p>                                                                           |                                                                                                                                                                          |
| <b>General issues</b>                                                                                                                                                                                                                                                                                                                                                                | <p>How does conversational AI fit into your everyday life?</p> <p>Walk me through the types of situations that usually make you open an AI tool.</p> <p>Do you use a specific language or tone when using chatbots? If so, what is the benefit?</p> <p>How do you choose between AI and other sources for health information?</p>                                                                                                                                                                                                                                                                                                                                                                                              | <p>Tour question p. 201.</p> <p>Walk me through questions. Comparison p. 202.</p> <p>Identify contrasts.</p> <p>Ask about something they know to ease the interview.</p> |
| <b>Deep focus: Health information seeking history</b>                                                                                                                                                                                                                                                                                                                                | <p>Tell me about the most recent time you asked an AI a health question.</p> <p>How did you feel about the response?</p> <p>Was there a specific reason you used AI in that moment?</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        | Grounded questions p. 202.                                                                                                                                               |
| <b>Scenario chatbot-based interaction</b>                                                                                                                                                                                                                                                                                                                                            |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                |                                                                                                                                                                          |
| <b>Deep focus: Follow-up</b>                                                                                                                                                                                                                                                                                                                                                         | <p>I noticed you chose to write [X]; what led you to word it that way?</p> <p>What did you like or dislike about the way it spoke to you versus the actual advice it gave?</p> <p>Is the chatbot emotionally supportive?</p> <p>Did the response meet your expectations?</p> <p>How do you feel about the AI's wording or phrasing of the health issues?</p> <p>Did anything surprise you?</p> <p>Were its answers useful to you?</p> <p>Talk us through how you used the AI.</p> <p>Because there are many possible ways of writing messages to the AI, why did you write these messages specifically?</p> <p>Is there anything in particular that you are mindful of when prompting, such as wording or how it responds?</p> | Probe questions and clarify answers p. 202.                                                                                                                              |
| <b>Retrospective questions</b>                                                                                                                                                                                                                                                                                                                                                       | What does your ideal AI response look like?                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    | Allows interviewee to reflect on broader issues after the deep focus.                                                                                                    |
| <b>Wrap up</b>                                                                                                                                                                                                                                                                                                                                                                       | <p>Is there anything we missed or you would like to add?</p> <p>Thank you for your time.</p> <p>How should we contact you if you win the raffle?</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           |                                                                                                                                                                          |
| <b>Keywords for follow-up, if relevant:</b> Chatbot technological understanding, comparing with other tools, trust, sycophancy, health anxiety, self-diagnosis, model differences, chatbot personality, assisted decision-making, views on chatbot use by self vs. by others, paid subscriptions to models, asking chatbot to respond a specific way, such as objectively or kindly. |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                |                                                                                                                                                                          |

**Table A.2: Participant Scenarios for Chatbot Interaction Tasks, Mental & physical.**

| Scenario A: Mental Well-being Focus                                                                                                                                                                                                                                                                        | Scenario B: Physical Well-being Focus                                                                                                                                                                                                                                                                                                                                                                                   |
|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Task Instructions handed to Participants:</b> <i>Please ask the chatbot about the symptoms in this scenario, similarly to how you have used AI before.</i>                                                                                                                                              |                                                                                                                                                                                                                                                                                                                                                                                                                         |
| <p><b>Core Scenario Text:</b><br/>         “You have been feeling stressed several times through the day for two weeks now. In that time, you have not been able to get a full night’s sleep most nights, even though nothing in your environment has seemingly changed.”</p>                              | <p><b>Core Scenario Text:</b><br/>         “For three weeks, you’ve felt under the weather. You wake up with heavy tension in your neck, have a hard time catching your breath, and have had difficulty focusing on everyday tasks.”</p>                                                                                                                                                                                |
| <p><b>Additional contextual details provided:</b></p> <ul style="list-style-type: none"> <li>• Waking up 2–3 times through the night.</li> <li>• Experiencing difficulty focusing during the day.</li> <li>• Increased caffeine intake over the past week (1–2 cups of coffee more than usual).</li> </ul> | <p><b>Additional contextual details provided:</b></p> <ul style="list-style-type: none"> <li>• Difficulty focusing manifests or feels like dizziness.</li> <li>• Sweating more than usual.</li> <li>• Tension can spread from the back of the head down to the shoulders.</li> <li>• Heart racing feels akin to having just completed a long run.</li> <li>• Morning headaches occur nearly every other day.</li> </ul> |